



SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) : 1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☒		Follow-up Case ☐		FOR AMC / NCC USE ONLY							
A. PATIENT INFORMATION *				Reg. No. / IPD No. / OPD No. / CR No. :							
1. Patient Initials ^{NIP}		2. Age ⁶² or date of birth:		AMC Report No. :							
3. Gender: M ☐ F ☒ Other ☐		4. Weight (in Kg.)		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION * 5. Event / Reaction start date (dd/mm/yyyy) After 10 days 6. Event / Reaction stop date (dd/mm/yyyy) 7. Describe Event/Reaction management with details, if any A 62-year-old female started Atorvastatin 20 mg OD for hyperlipidemia and developed muscle pain and weakness after 10 days. Symptoms subsided after discontinuation. No hospitalization required.				12. Relevant investigations with dates :							
				13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)							
				14. Seriousness ^X of the reaction : No ☐ if Yes ☐ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☐ Hospitalization-Initial/Prolonged ☐ Other Medically important							
				15. Outcome: ☒ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
C. SUSPECTED MEDICATION(S) *											
S. No.	8. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Atorvastatin	20 mg			Oral	OD				hyperlipidemia	
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i	X										
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i											
ii											
iii [#]											
Additional Information :						D. REPORTER DETAILS *					
Simulated Case 1 — Non-serious - Expected						16. Name & Address : Chandranath Dey Guru Nanak Institute of Pharmaceutical Science and Technology 157/F Nilgunj Road, Panihati, Kolkata - 700114, West Bengal, India Pin : _____ Email : _____ Contact No- : _____ Occupation : _____ Signature : _____					
						17. Date of this report (dd/mm/yyyy) :					
Signature and Name of Receiving Personnel :											
Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.											

[#] Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

ADVICE ABOUT REPORTING

A. What to report?

All adverse events should be reported

Report non-serious, known or unknown, frequent or rare adverse drug reactions due to Medicines, Vaccines & Herbal Products.

Report every serious adverse drug reactions. A reaction is serious when the patient outcome is :

- Death
- Life-threatening
- Hospitalization (initial or prolonged)
- Disability (significant, persistent or permanent)
- Congenital anomaly
- Report intervention to prevent permanent impairment or damage

NOTE : Serious/Adverse Event following immunization can also be reported in Serious AEFI case Notification Form available on <http://www.ipc.gov.in>

B. Who can report?

All healthcare professionals (Clinicians, Dentists, Pharmacists and Nurse etc.) can report adverse drug reactions

C. Where to report?

Duly filled in Suspected Adverse Drug Reaction Reporting Form can be sent to the nearest Adverse Drug Reaction Monitoring Centre (AMC) or directly to the National Coordination Centre (NCC) for PvPI.

Call on Helpline (Toll Free) 1800 180 3024 to report ADRs or directly mail this filled form to pvpi.ipc@gov.in

A list of nationwide AMCs is available at : <http://www.ipc.gov.in>, http://www.ipc.gov.in/PvPI/pv_home.html

D.What happens to the submitted information?

- Information provided in this form is handled in strict confidence. The causality assessment is carried out at AMCs by using WHO-UMC scale. The analyzed forms are forwarded to the NCC-PvPI through ADR database. Finally the data is analyzed and forwarded to the Global Pharmacovigilance Database managed by WHO Uppsala Monitoring Centre in Sweden.
- The reports are periodically reviewed by the NCC-PvPI. The information generated on the basis of these reports helps in continuous assessment of the benefit-risk ratio of medicines.
- The Signal Review Panel of PvPI reviews the data and suggests any interventions that may be required.

E. Mandatory fields for suspected ADR Reporting Form (*)

Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

For Adverse Drug Reaction Reporting Tools

- E-mail : pvpi.ipc@gov.in
- PvPI Helpline (Toll Free) : 1800 180 3024 (9:00 AM to 5:30 PM, Monday-Friday)
- ADR Mobile App : "ADRPvPI"



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Initial Case ☒		Follow-up Case ☐		FOR AMC / NCC USE ONLY							
A. PATIENT INFORMATION *				Reg. No. / IPD No. / OPD No. / CR No. :							
1. Patient Initials ^{NIP}		2. Age ³⁴ or date of birth:		AMC Report No. :							
3. Gender: M ☒ F ☐ Other ☐		4. Weight (in Kg.)		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION * 5. Event / Reaction start date (dd/mm/yyyy) On 3rd day 6. Event / Reaction stop date (dd/mm/yyyy) 7. Describe Event/Reaction management with details, if any A 34-year-old male took Amoxicillin 500 mg TDS for tonsillitis and developed widespread skin rash and itching on 3rd day. Drug stopped. Symptoms improved with antihistamine. No hospitalization required.				12. Relevant investigations with dates :							
				13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)							
				14. Seriousness ^x of the reaction : No ☐ if Yes ☐ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☐ Hospitalization-Initial/Prolonged ☐ Other Medically important							
				15. Outcome: ☒ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
C. SUSPECTED MEDICATION(S) *											
S. No.	8. Name (Brand / Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Amoxicillin	500 mg			Oral	TDS				Tonsillitis	
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i	X										
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i											
ii											
iii [#]											
Additional Information :						D. REPORTER DETAILS *					
Simulated Case 2 — Non-serious - Expected						16. Name & Address : Chandranath Dey Guru Nanak Institute of Pharmaceutical Science and Technology 15/7F Nilgunj Road, Panihati, Kolkata - 700114, West Bengal, India Pin : _____ Email : _____ Contact No- : _____ Occupation : _____ Signature : _____					
						17. Date of this report (dd/mm/yyyy) :					
Signature and Name of Receiving Personnel :											
Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.											

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- Death
- Life-threatening
- Hospitalization (initial or prolonged)
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- Congenital anomaly
- Report intervention to prevent permanent impairment or damage

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C. Where to report?

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D.What happens to the submitted information?

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- The reports are periodically reviewed by the NCC-PvPI. The information generated on the basis of these reports helps in continuous assessment of the benefit-risk ratio of medicines.
- The Signal Review Panel of PvPI reviews the data and suggests any interventions that may be required.

E. Mandatory fields for suspected ADR Reporting Form (*)

Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

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1. Patient Initials ^{NIP}		2. Age ⁴⁵ or date of birth:		AMC Report No. :							
3. Gender: M ☒ F ☐ Other ☐		4. Weight (in Kg.)		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION * 5. Event / Reaction start date (dd/mm/yyyy) Not provided 6. Event / Reaction stop date (dd/mm/yyyy) 7. Describe Event/Reaction management with details, if any A 45-year-old male on Diclofenac 50 mg TDS for back pain developed severe gastric pain and black stools. Hospitalization required. Drug discontinued. Recovered after treatment.				12. Relevant investigations with dates :							
				13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)							
				14. Seriousness of the reaction : No ☐ if Yes ☒ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☒ Hospitalization-Initial/Prolonged ☐ Other Medically important							
				15. Outcome: ☒ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
C. SUSPECTED MEDICATION(S) *											
S. No.	8. Name (Brand / Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Diclofenac	50 mg			Oral	TDS				back pain	
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i	X										
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i											
ii											
iii [#]											
Additional Information :						D. REPORTER DETAILS *					
Simulated Case 3 — Serious - Expected						16. Name & Address : Chandranath Dey Guru Nanak Institute of Pharmaceutical Science and Technology 15/7F Nilgunj Road, Panihati, Kolkata - 700114, West Bengal, India Pin : _____ Email : _____ Contact No- : _____ Occupation : _____ Signature : _____					
						17. Date of this report (dd/mm/yyyy) :					
Signature and Name of Receiving Personnel :											
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D.What happens to the submitted information?

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Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

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Initial Case ☒		Follow-up Case ☐		FOR AMC / NCC USE ONLY							
A. PATIENT INFORMATION *				Reg. No. / IPD No. / OPD No. / CR No. :							
1. Patient Initials ^{NIP}		2. Age ⁵⁸ or date of birth:		AMC Report No. :							
3. Gender: M ☐ F ☒ Other ☐		4. Weight (in Kg.)		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION *				12. Relevant investigations with dates :							
5. Event / Reaction start date (dd/mm/yyyy) Not provided				13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)							
6. Event / Reaction stop date (dd/mm/yyyy)											
7. Describe Event/Reaction management with details, if any A 58-year-old female on Levofloxacin 500 mg OD for UTI developed palpitations and fainting episode. Life-threatening condition. ICU admission required. Recovered after treatment.											
				14. Seriousness of the reaction : No ☐ if Yes ☒ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☒ Life threatening ☐ Disability ☐ Hospitalization-Initial/Prolonged ☐ Other Medically important							
				15. Outcome: ☒ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
C. SUSPECTED MEDICATION(S) *											
S. No.	8. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Levofloxacin	500 mg			Oral	OD				UTI	
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i	X										
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i											
ii											
iii [#]											
Additional Information : Simulated Case 4 — Serious - Unexpected						D. REPORTER DETAILS *					
						16. Name & Address : Chandranath Dey Guru Nanak Institute of Pharmaceutical Science and Technology 157/F Nilgunj Road, Panihati, Kolkata - 700114, West Bengal, India Pin : _____ Email : _____ Contact No- : _____ Occupation : _____ Signature : _____					
						17. Date of this report (dd/mm/yyyy) :					
Signature and Name of Receiving Personnel :											
Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.											

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ADVICE ABOUT REPORTING

A. What to report?

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- Hospitalization (initial or prolonged)
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D.What happens to the submitted information?

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3. Gender: M ☒ F ☐ Other ☐		4. Weight (in Kg.)		Worldwide Unique No. :							
B. SUSPECTED ADVERSE REACTION * 5. Event / Reaction start date (dd/mm/yyyy) In first week 6. Event / Reaction stop date (dd/mm/yyyy) 7. Describe Event/Reaction management with details, if any A 29-year-old male taking Metformin 500 mg BD for diabetes developed nausea and vomiting in the first week. Symptoms mild and resolved spontaneously. No treatment required.				12. Relevant investigations with dates :							
				13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)							
				14. Seriousness ^x of the reaction : No ☐ if Yes ☐ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☐ Hospitalization-Initial/Prolonged ☐ Other Medically important							
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S. No.	8. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment
								Date Started	Date Stopped		
i	Metformin	500 mg			Oral	BD				Diabetes	
ii											
iii											
iv [#]											
9. Action taken after reaction (please tick)								10. Reaction reappeared after reintroduction of suspected medication (please tick)			
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)	
i				X							
ii											
iii											
iv											
11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)											
S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates		Indication				
					Date Started	Date Stopped					
i											
ii											
iii [#]											
Additional Information :						D. REPORTER DETAILS *					
Simulated Case 5 — Non-serious - Expected						16. Name & Address : Chandranath Dey Guru Nanak Institute of Pharmaceutical Science and Technology 157/F Nilgunj Road, Panihati, Kolkata - 700114, West Bengal, India Pin : _____ Email : _____ Contact No- : _____ Occupation : _____ Signature : _____					
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A list of nationwide AMCs is available at : <http://www.ipc.gov.in>, http://www.ipc.gov.in/PvPI/pv_home.html

D.What happens to the submitted information?

- Information provided in this form is handled in strict confidence. The causality assessment is carried out at AMCs by using WHO-UMC scale. The analyzed forms are forwarded to the NCC-PvPI through ADR database. Finally the data is analyzed and forwarded to the Global Pharmacovigilance Database managed by WHO Uppsala Monitoring Centre in Sweden.
- The reports are periodically reviewed by the NCC-PvPI. The information generated on the basis of these reports helps in continuous assessment of the benefit-risk ratio of medicines.
- The Signal Review Panel of PvPI reviews the data and suggests any interventions that may be required.

E. Mandatory fields for suspected ADR Reporting Form (*)

Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

For Adverse Drug Reaction Reporting Tools

- E-mail : pvpi.ipc@gov.in
- PvPI Helpline (Toll Free) : 1800 180 3024 (9:00 AM to 5:30 PM, Monday-Friday)
- ADR Mobile App : "ADRPvPI"